

Stroke Patient Escalation Protocol

Stroke Program - Clinical Escalation

Purpose

To provide a standardized response when a stroke patient clinically deteriorates.

General Escalation Flow

Clinical Deterioration
?
Primary Nurse
?
Notify Stroke Physician Immediately
?
Rapid Assessment
?
Determine Cause
?
Appropriate Pathway

Deterioration-Specific Pathways

Neurological Deterioration

Examples: NIHSS increase ≥ 4 · GCS decline · new weakness · new aphasia

Immediate physician review

STAT CT Brain

Consider CTA

Notify neurosurgery

ICU transfer if needed

Hemorrhagic Transformation

Stop thrombolytic/anticoagulant if applicable

Repeat CT

Reverse anticoagulation if indicated

Neurosurgery consult

ICU admission

Airway Compromise

Respiratory therapy

Anesthesia

Intubation

ICU

Seizures

Notify stroke physician

Lorazepam

Antiepileptic loading

EEG

Neurology review

Blood Pressure Crisis

Follow IV antihypertensive protocol

Frequent monitoring

Escalate to ICU if uncontrolled

Cerebral Edema

Repeat CT

Hyperosmolar therapy

Neurosurgery consult

ICU

Cardiac Complications

ECG

Troponin

Cardiology consult

Telemetry

Documentation

All escalations must be documented in:

Progress notes

Stroke flowsheet

Escalation record

ICU transfer note (if applicable)